



GV-GB-001

ACTIVE

REQUIRED

Governing Body Authority & Responsibilities

VERSION	EFFECTIVE
v6.0	2025-07-10
LAST REVIEWED	NEXT REVIEW
2025-07-10	2026-07-10
POLICY OWNER	SUBDOMAIN
Administrator	GB – Governing Body
DOMAIN	
GV – Governance & Administration	
APPROVED BY	
Governing Body Chair	

 2. PURPOSE

This policy establishes the authority, composition, functions, and oversight responsibilities of the Governing Body of Care Indeed Home Health Care, Inc. The Governing Body is the ultimate authority accountable for the operation, management, fiscal viability, and regulatory compliance of the home health agency. This policy ensures the agency satisfies the requirements set forth in 42 CFR § 484.105 – Condition of Participation: Organization and Administration of Services, which mandates that a home health agency must have a governing body that assumes full legal authority and responsibility for the agency's overall operation and management.

3. SCOPE

This policy applies to:

- ✓ All members of the Governing Body of Care Indeed Home Health Care, Inc. (including voting and non-voting members)
- ✓ The Agency Administrator
- ✓ The Director of Nursing / Clinical Manager
- ✓ The Compliance Officer
- ✓ All senior leadership personnel who report directly to the Governing Body or Administrator
- ✓ All contracted management entities performing governing body functions on behalf of the agency

This policy does not apply to day-to-day clinical or operational staff except to the extent that Governing Body decisions establish requirements, standards, or directives that govern their work.

4. POLICY STATEMENT

- 4.1** Care Indeed Home Health Care, Inc. shall maintain a designated Governing Body that holds full legal authority and responsibility for the overall operation, management, and regulatory compliance of the home health agency, as required by 42 CFR § 484.105(a).
- 4.2** The Governing Body shall be responsible for ensuring that Care Indeed Home Health Care, Inc. operates in compliance with all applicable federal, state, and local laws, regulations, and licensure requirements at all times.
- 4.3** The Governing Body shall appoint a qualified Administrator who is authorized to act on behalf of the Governing Body in the day-to-day management of the agency and who meets the qualifications defined in agency policy GV-OG-002 and applicable California state requirements.
- 4.4** The Governing Body shall ensure the appointment and ongoing oversight of a qualified Clinical Manager (Director of Nursing) responsible for all clinical services, in compliance with 42 CFR § 484.105(c).
- 4.5** The Governing Body shall approve and oversee the agency's:
- Scope of services (GV-OG-003)
 - Organizational structure and reporting lines (GV-OG-001)
 - Annual strategic plan and operational goals (GV-OG-004)
 - Policy framework and all REQUIRED-tier policies (GV-PM-001, GV-PM-002)
 - QAPI program (QA-PG-001, QA-PG-002)
 - Corporate compliance program (CO-CP-001)
 - Annual operating budget (FN-FP-005)
 - Emergency preparedness plan (OP-FM-005)
- 4.6** The Governing Body shall meet at a frequency sufficient to fulfill its oversight responsibilities, but not less than quarterly, with meetings documented in formal minutes per policy GV-GB-002.
- 4.7** The Governing Body shall not delegate its ultimate accountability for regulatory compliance, patient safety, or fiscal integrity. Delegation of specific authority shall comply with policy GV-OG-005.
- 4.8** All members of the Governing Body shall disclose and manage conflicts of interest in accordance with policy GV-GB-003.
- 4.9** Only the most current approved version of this policy shall be considered valid. Superseded versions must not be used for any operational or compliance purpose. Any revision requires re-acknowledgment by all Governing Body members and senior leadership within 14 calendar days of the revised effective date.

5. DEFINITIONS

GOVERNING BODY

The individual(s), board of directors, trustees, partnership, corporation, or other legally constituted authority that has ultimate responsibility for the management and operation of Care Indeed Home Health Care, Inc., as defined by 42 CFR § 484.2 and § 484.105.

CLINICAL MANAGER

The registered nurse (or qualified individual per California state law) designated by the Governing Body to oversee clinical services including patient care delivery, clinical staff supervision, and OASIS compliance. Also referred to as Director of Nursing (DON).

QUORUM

The minimum number of Governing Body members required to be present (physically or via approved teleconference) to conduct official business, as defined in the agency's bylaws or operating agreement.

ADMINISTRATOR

The individual appointed by the Governing Body who is responsible for managing the agency's day-to-day operations and who meets all qualifications specified in agency policy GV-OG-002 and applicable California state law.

FIDUCIARY DUTY

The legal obligation of Governing Body members to act in good faith, with due diligence, and in the best interest of the agency and the patients it serves.

QAPI

Quality Assessment and Performance Improvement — the structured program required by 42 CFR § 484.65 for ongoing quality monitoring and improvement.



6.1 ESTABLISHMENT AND COMPOSITION

STEP	RESPONSIBLE PARTY	ACTION	TIMEFRAME
6.1.1	Agency Owner / Corporate Entity	Formally establish the Governing Body through articles of incorporation, operating agreement, partnership agreement, or equivalent legal instrument for Care Indeed Home Health Care, Inc. The establishing document must identify: (a) the legal form of the Governing Body; (b) the minimum and maximum number of members; (c) the quorum requirement; (d) the terms of appointment or election.	Prior to initial Medicare certification and maintained continuously thereafter.
6.1.2	Governing Body Chair	Maintain a current roster of all Governing Body members including: full legal name, title/role, date of appointment, term expiration date, voting status, and contact information.	Updated within 7 calendar days of any membership change.

STEP	RESPONSIBLE PARTY	ACTION	TIMEFRAME
6.1.3	Governing Body	Ensure composition includes, at minimum, individuals with competency in: (a) healthcare operations or clinical services; (b) financial management; (c) regulatory compliance. If any competency area is not represented by a current member, the Governing Body shall retain qualified advisory counsel within 30 calendar days of identifying the gap.	Ongoing; reviewed annually at the first quarterly meeting of each calendar year.
6.1.4	Compliance Officer	Verify that no Governing Body member appears on the OIG List of Excluded Individuals/Entities (LEIE) or the System for Award Management (SAM) exclusion database at the time of appointment and monthly thereafter, per policy HR-TA-003.	At appointment and monthly thereafter.

 6.2 CORE RESPONSIBILITIES

The Governing Body of Care Indeed Home Health Care, Inc. shall fulfill the following responsibilities directly and shall not delegate ultimate accountability for any of these functions.

6.2.1 — Legal Authority and Agency Operations

STEP	RESPONSIBLE PARTY	ACTION	TIMEFRAME
6.2.1.1	Governing Body	Assume and maintain full legal authority for the overall operation, management, and fiscal viability of Care Indeed Home Health Care, Inc.	Continuous.
6.2.1.2	Governing Body	Ensure the agency maintains current and valid: (a) California home health license – HCAI License No. 406412878; (b) Medicare certification; (c) Medicaid enrollment (if applicable); (d) accreditation (if applicable) – per policy GV-EA-004.	Continuous; verified at each quarterly meeting.
6.2.1.3	Governing Body	Review and approve the agency's defined scope of services (policy GV-OG-003) at least annually. Ensure the agency does not provide services beyond those for which it is licensed, staffed, and competent to deliver.	Annually, within 30 calendar days of the start of the fiscal year.

6.2.2 — Appointment and Oversight of Key Personnel

STEP	RESPONSIBLE PARTY	ACTION	TIMEFRAME
6.2.2.1	Governing Body	Appoint a qualified Administrator and document the appointment in Governing Body minutes. The Administrator must meet all qualifications defined in policy GV-OG-002 and applicable California state law.	Prior to agency operation and within 30 calendar days of any vacancy.
6.2.2.2	Governing Body	Appoint or confirm the appointment of a qualified Clinical Manager (Director of Nursing) responsible for all clinical services, per 42 CFR § 484.105(c).	Prior to agency operation and within 30 calendar days of any vacancy.
6.2.2.3	Governing Body	Appoint or confirm the designation of a Compliance Officer with authority and independence to operate the corporate compliance program, per policy CO-CP-002.	Prior to agency operation and within 30 calendar days of any vacancy.
6.2.2.4	Governing Body	Conduct or commission an annual performance evaluation of the Administrator. Results and any corrective directives shall be documented in executive session minutes.	Annually; completed within 60 calendar days of the end of each fiscal year.

STEP	RESPONSIBLE PARTY	ACTION	TIMEFRAME
6.2.2.5	Governing Body	Review and approve the agency's succession plan for the Administrator, Clinical Manager, and Compliance Officer, per policy GV-GB-004.	Annually at the second quarterly meeting; updated within 14 calendar days of any key leadership vacancy.

6.2.3 — Policy and Compliance Oversight

STEP	RESPONSIBLE PARTY	ACTION	TIMEFRAME
6.2.3.1	Governing Body	Approve all REQUIRED-tier policies prior to implementation and ensure a defined policy review cycle exists per policies GV-PM-001 and GV-PM-002.	Prior to implementation of each REQUIRED policy; review cycle approved annually.
6.2.3.2	Governing Body	Receive and review a compliance status report from the Compliance Officer at each quarterly meeting. The report must address: (a) active compliance investigations; (b) audit findings; (c) regulatory changes affecting the agency; (d) training completion rates.	Quarterly.
6.2.3.3	Compliance Officer	Prepare and submit the quarterly compliance report to the Governing Body no fewer than 7 calendar days before each scheduled quarterly meeting.	7 calendar days before each quarterly meeting.
6.2.3.4	Governing Body	Review and act upon any compliance deficiency identified as high-risk within 14 calendar days of receiving the compliance report. Action must be documented in meeting minutes or a special resolution.	Within 14 calendar days of report receipt.

6.2.4 — QAPI Oversight

STEP	RESPONSIBLE PARTY	ACTION	TIMEFRAME
6.2.4.1	Governing Body	Approve the agency's QAPI plan, per policy QA-PG-002, and ensure that the plan includes measurable quality indicators, performance improvement projects, and patient safety initiatives.	Annually; approved at the first quarterly meeting of each calendar year.
6.2.4.2	Clinical Manager / QA Designee	Present a QAPI performance report to the Governing Body at each quarterly meeting including: (a) quality indicator trends; (b) status of active PIPs; (c) adverse event summary; (d) patient satisfaction data; (e) Star Rating / Home Health Compare trends (policy QA-SM-004).	Quarterly.
6.2.4.3	Governing Body	Review, discuss, and document its response to the QAPI report. If any quality indicator falls below the defined threshold for 2 consecutive reporting periods, the Governing Body shall direct corrective action and assign accountability with a defined resolution deadline.	At each quarterly meeting; corrective action directive within 14 calendar days if thresholds are breached.

6.2.5 — Financial Oversight

STEP	RESPONSIBLE PARTY	ACTION	TIMEFRAME
6.2.5.1	Governing Body	Review and approve the annual operating budget per policy FN-FP-005.	Annually; approved no later than 30 calendar days before the start of each fiscal year.
6.2.5.2	Administrator	Present a financial performance report to the Governing Body at each quarterly meeting including: (a) revenue vs. budget variance; (b) accounts receivable aging; (c) claims denial rate and trending; (d) cash flow position.	Quarterly.
6.2.5.3	Governing Body	Review financial reports and direct corrective action if: (a) actual revenue deviates more than 10% below budget for 2 consecutive months; (b) claims denial rate exceeds 5%; (c) days in accounts receivable exceed 60. Directives must be documented in meeting minutes.	At each quarterly meeting.

6.2.6 — Emergency Preparedness

STEP	RESPONSIBLE PARTY	ACTION	TIMEFRAME
6.2.6.1	Governing Body	Approve the Emergency Operations and Business Continuity Plan per policy OP-FM-005 and 42 CFR § 484.102.	Annually; approved at the third quarterly meeting.
6.2.6.2	Administrator / Governing Body	Report the results of the most recent emergency preparedness drill or exercise to the Governing Body, including identified gaps and corrective actions; the Governing Body shall review results and direct corrective action as needed per policy OP-FM-005.	At the quarterly meeting following each drill (minimum 2 drills per year).

 6.3 GOVERNING BODY MEETINGS

STEP	RESPONSIBLE PARTY	ACTION	TIMEFRAME
6.3.1	Governing Body Chair	Schedule and convene regular Governing Body meetings no fewer than 4 times per calendar year (quarterly). The meeting schedule for the upcoming year must be established and distributed to all members by December 15 of the preceding year.	Quarterly; schedule distributed by December 15.
6.3.2	Governing Body Chair	Convene special meetings when urgent matters arise, including but not limited to: (a) CMS survey findings requiring immediate corrective action; (b) serious adverse events; (c) regulatory enforcement actions; (d) key leadership vacancies. Notice of a special meeting must be provided to all members at least 48 hours in advance unless the matter constitutes an imminent threat to patient safety, in which case notice may be shortened to the minimum practicable.	As needed; notice within 48 hours or shorter for imminent patient safety threats.

STEP	RESPONSIBLE PARTY	ACTION	TIMEFRAME
6.3.3	Designated Secretary / Administrator	Prepare and distribute the meeting agenda to all Governing Body members no fewer than 7 calendar days before each scheduled meeting. The agenda must include standing items: (a) approval of prior minutes; (b) Administrator report; (c) compliance report; (d) QAPI report; (e) financial report; (f) old business; (g) new business.	7 calendar days before each meeting.
6.3.4	Designated Secretary	Record formal minutes for each meeting per policy GV-GB-002. Minutes must document: (a) date, time, and location; (b) members present and absent; (c) quorum determination; (d) all motions, seconds, and voting outcomes; (e) all directives issued with assigned responsible parties and deadlines; (f) executive session topics (without protected details).	Draft minutes completed within 14 calendar days of the meeting; approved at the next regular meeting.
6.3.5	Governing Body Chair	Ensure a quorum is present before conducting any official business. If quorum is not achieved, the meeting shall be rescheduled within 14 calendar days.	At the start of each meeting.



6.4 CONFLICT OF INTEREST MANAGEMENT

STEP	RESPONSIBLE PARTY	ACTION	TIMEFRAME
6.4.1	All Governing Body Members	Complete and submit the Conflict of Interest Disclosure Form (Appendix B) at the time of appointment, annually thereafter, and within 7 calendar days of any change in circumstances that could create a new conflict, per policy GV-GB-003.	At appointment; annually; within 7 days of change.
6.4.2	Compliance Officer	Review all submitted conflict of interest disclosures within 14 calendar days of receipt and present a summary to the Governing Body with recommendations for management or recusal.	Within 14 calendar days of receipt.
6.4.3	Governing Body	Act on conflict of interest recommendations. Any member with a disclosed conflict shall recuse from discussion and voting on the affected matter. Recusals must be documented in meeting minutes.	At the meeting where the affected matter is addressed.

⚠ 6.5 Escalation and Exception Handling

CONDITION	ESCALATION PATH	CORRECTIVE ACTION	TIMEFRAME
Governing Body fails to meet quarterly	Administrator notifies all members and the Compliance Officer in writing.	Administrator schedules a make-up meeting. If Governing Body does not convene within 30 calendar days, the Compliance Officer documents the deficiency and initiates corrective action per QA-AE-003.	Make-up meeting within 30 calendar days of the missed quarter.
Quorum not achieved for 2 consecutive scheduled meetings	Governing Body Chair escalates to the full membership in writing.	Chair initiates membership recruitment or replacement per the agency's bylaws. Issue must be resolved before the next scheduled meeting.	Within 30 calendar days.
Key leadership vacancy (Administrator, Clinical Manager, Compliance Officer) exceeds 30 days unfilled	Governing Body Chair	Governing Body must appoint an interim designee within 14 calendar days of vacancy and document the appointment. Permanent appointment must occur within 90 calendar days.	Interim: 14 days. Permanent: 90 days.
CMS survey results in Condition-level deficiency	Administrator convenes a special Governing Body meeting.	Governing Body directs development of a Plan of Correction within CMS-required timeframes (typically 10 calendar days). Governing Body receives weekly status updates until resolution is confirmed.	Special meeting within 48 hours of receipt of findings; Plan of Correction per CMS deadline.

CONDITION	ESCALATION PATH	CORRECTIVE ACTION	TIMEFRAME
Compliance Officer reports fraud, waste, or abuse concern to Governing Body	Governing Body Chair	Governing Body directs investigation per CO-CP-007 and ensures non-retaliation per CO-CP-005. Governing Body receives investigation status updates at each meeting until resolution.	Investigation initiated within 7 calendar days; updates at each meeting.



7. DOCUMENTATION REQUIREMENTS

REQUIREMENT	DOCUMENT / RECORD	RESPONSIBLE PARTY	LOCATION	TIMEFRAME
Governing Body establishment	Articles of incorporation, operating agreement, bylaws, or equivalent legal instrument establishing the Governing Body of Care Indeed Home Health Care, Inc.	Agency Owner / Corporate Entity	Corporate records repository (physical or electronic).	Maintained permanently; updated within 14 calendar days of any amendment.
Governing Body membership roster	Current roster including member name, role, appointment date, term, voting status, and contact information (Appendix A).	Governing Body Chair	Agency governance file; copy maintained by Administrator.	Updated within 7 calendar days of any change.
Meeting minutes	Formal minutes for all regular and special meetings, per policy GV-GB-002 (Appendix D template).	Designated Secretary	Agency governance file; copy provided to each member.	Draft within 14 calendar days of meeting; approved at next regular meeting. Retained for minimum 7 years.
Meeting agendas	Agenda for each regular and special meeting.	Administrator / Designated Secretary	Agency governance file.	Distributed 7 calendar days before each meeting; retained for minimum 7 years.

REQUIREMENT	DOCUMENT / RECORD	RESPONSIBLE PARTY	LOCATION	TIMEFRAME
Administrator appointment	Written documentation of the Governing Body's appointment of the Administrator, including qualifications verified.	Governing Body Chair	Governing Body minutes; Administrator personnel file.	At time of appointment.
Clinical Manager appointment	Written documentation of the appointment or confirmation of the Clinical Manager.	Governing Body Chair	Governing Body minutes; Clinical Manager personnel file.	At time of appointment.
Compliance Officer designation	Written documentation of the designation and authority granted to the Compliance Officer.	Governing Body Chair	Governing Body minutes; Compliance Officer personnel file.	At time of designation.
Conflict of Interest disclosures	Completed Conflict of Interest Disclosure Forms (Appendix B) for each Governing Body member.	Compliance Officer (collection); each member (completion)	Compliance file; copy in governance file.	At appointment; annually; within 7 days of change. Retained for minimum 7 years.
Quarterly compliance reports	Compliance Officer's written report to the Governing Body.	Compliance Officer	Agency governance file; compliance records.	Submitted 7 days before each quarterly meeting; retained minimum 7 years.
Quarterly QAPI reports	Clinical Manager / QA Designee's written report to the Governing Body.	Clinical Manager / QA Designee	Agency governance file; QAPI records.	Presented at each quarterly meeting; retained minimum 7 years.

REQUIREMENT	DOCUMENT / RECORD	RESPONSIBLE PARTY	LOCATION	TIMEFRAME
Financial reports	Quarterly financial performance report presented to the Governing Body.	Administrator	Agency governance file; financial records.	Presented at each quarterly meeting; retained per CO-HP-007.
Annual budget approval	Documented Governing Body approval of the annual operating budget.	Administrator (preparation); Governing Body (approval)	Governing Body minutes; financial records.	Annually; approved 30 days before fiscal year start.
QAPI plan approval	Documented Governing Body approval of the annual QAPI plan.	Clinical Manager / QA Designee (preparation); Governing Body (approval)	Governing Body minutes; QAPI records.	Annually at first quarterly meeting.
Emergency preparedness plan approval	Documented Governing Body approval of the emergency plan.	Administrator (preparation); Governing Body (approval)	Governing Body minutes; emergency preparedness file.	Annually at third quarterly meeting.
Exclusion screening results	Documentation of OIG/SAM screening for each Governing Body member.	Compliance Officer	Compliance file.	At appointment; monthly thereafter.
Policy acknowledgment	Signed acknowledgment of this policy by all Governing Body members and senior leadership (Appendix C).	Each member / leader (completion); Administrator (collection)	Policy acknowledgment file.	Within 14 calendar days of policy effective date or revision; within 14 calendar days of new appointment.
Annual Governance Self-Assessment	Completed self-assessment per policy GV-GB-005 (if adopted).	Governing Body Chair	Governance file.	Annually.

**8. COMPLIANCE & AUDIT****8.1 How Compliance Is Measured**

COMPLIANCE INDICATOR	MEASUREMENT METHOD	ACCEPTABLE STANDARD
Governing Body is legally established and documented.	Review of establishing documents (articles, bylaws, operating agreement).	Current, complete, and on file at all times.
Governing Body meets at least quarterly.	Review of meeting minutes with dates and attendance.	4 or more meetings per calendar year with quorum present at each.
Key personnel (Administrator, Clinical Manager, Compliance Officer) are appointed and documented.	Review of Governing Body minutes; personnel files; appointment letters.	Current appointments documented; no vacancy exceeds 30 days without interim designee.
QAPI plan is reviewed and approved annually.	Review of Governing Body minutes for documented approval.	Annual approval documented at first quarterly meeting.
Compliance reports are presented quarterly.	Review of Governing Body agendas and minutes; compliance report files.	Reports submitted 7 days before each meeting; discussion documented in minutes.
Conflict of Interest disclosures are current.	Review of Appendix B forms for each member; annual renewal dates.	100% completion rate; no lapsed disclosures.
Budget is approved annually.	Review of Governing Body minutes; budget document.	Approved no later than 30 days before fiscal year start.
Governing Body members are screened for exclusion.	Review of OIG/SAM screening logs.	Initial screening at appointment; monthly screening documented.
Policy acknowledgments are current.	Review of signed Appendix C forms.	100% acknowledgment within 14 calendar days of effective date or new appointment.

8.2 SURVEYOR EXPECTATIONS

CMS surveyors conducting a standard survey under SOM Appendix B will specifically verify:

1. Evidence that a Governing Body exists and is functioning. Surveyors will request establishing documents and a current membership roster. A sole proprietor must demonstrate individual acceptance of governing body responsibilities.
2. Evidence that the Governing Body has appointed a qualified Administrator. Surveyors will review Governing Body minutes for appointment documentation and verify qualifications per California state requirements.
3. Evidence of Clinical Manager oversight. Surveyors will look for Governing Body minutes documenting appointment, reporting, and oversight of clinical services leadership.
4. Evidence that the Governing Body oversees QAPI. Surveyors will examine whether the Governing Body has reviewed, approved, and acted upon quality data. Passive receipt of reports without documented action is a common deficiency.
5. Evidence of policy oversight. Surveyors will verify that the Governing Body has approved the agency's policies and that a review cycle exists.
6. Evidence of fiscal oversight. Surveyors will review whether the Governing Body monitors the agency's financial viability and acts on adverse trends.
7. Meeting frequency and documentation quality. Surveyors will request all meeting minutes for the survey look-back period and assess completeness, including attendance, quorum, and documented decisions.



8.3 COMMON FAILURE POINTS

NO DOCUMENTED EVIDENCE THAT A GOVERNING BODY EXISTS OR FUNCTIONS.

Risk: Condition-level deficiency under 42 CFR § 484.105. Potential termination of Medicare certification.

Mitigation: Maintain establishing documents, current roster, and quarterly minutes on file and readily accessible.

GOVERNING BODY MEETINGS ARE HELD BUT NOT DOCUMENTED.

Risk: Surveyor will treat undocumented meetings as not having occurred.

Mitigation: Use Appendix D template; complete draft minutes within 14 calendar days.

GOVERNING BODY 'RUBBER STAMPS' REPORTS WITHOUT DOCUMENTED DISCUSSION OR ACTION.

Risk: Surveyors will cite passive governance as failure to exercise oversight.

Mitigation: Minutes must document specific discussion points, questions, directives, and assigned follow-up.

KEY LEADERSHIP VACANCIES ARE UNFILLED FOR EXTENDED PERIODS.

Risk: Surveyor will cite failure to ensure adequate management.

Mitigation: Fill or designate interim within 14 calendar days per Section 6.5.

NO DOCUMENTED CONFLICT OF INTEREST DISCLOSURES.

Risk: OIG compliance program requirement; potential survey finding.

Mitigation: Enforce annual disclosure per Appendix B; Compliance Officer tracks compliance.

QAPI PLAN APPROVED BUT NO EVIDENCE OF GOVERNING BODY REVIEW OF QUALITY DATA.

Risk: Surveyor will cite failure of governing body oversight of QAPI (42 CFR § 484.65 cross-reference).

Mitigation: Require quarterly QAPI report presentation and document Governing Body response.



9. REFERENCES & ADMINISTRATION

9.1 Federal Regulations (42 CFR Part 484)

CITATION	TITLE	RELEVANCE
42 CFR § 484.2	Definitions	Defines 'governing body' and key terms for home health agencies.
42 CFR § 484.105	CoP: Organization and Administration of Services	Primary regulatory basis for this policy. Requires a governing body with full legal authority for agency operation and management.
42 CFR § 484.105(a)	Standard: Governing body	Mandates governing body responsibility for agency operations, appointment of administrator, and oversight of services.
42 CFR § 484.105(b)	Standard: Administrator	Requires appointment of a qualified administrator responsible to the governing body.
42 CFR § 484.105(c)	Standard: Clinical manager	Requires designation of a qualified clinical manager for oversight of clinical services.
42 CFR § 484.60	CoP: Care planning, coordination, and quality of care	Governing body accountability for ensuring care planning and quality.
42 CFR § 484.65	CoP: Quality assessment and performance improvement (QAPI)	Governing body must ensure an effective QAPI program.
42 CFR § 484.70	CoP: Infection prevention and control	Governing body oversight of infection prevention.
42 CFR § 484.100	CoP: Compliance with Federal, State, and local laws	Governing body must ensure full legal compliance.
42 CFR § 484.102	CoP: Emergency preparedness	Governing body must approve emergency preparedness plan.
42 CFR § 484.110	CoP: Clinical records	Governing body oversight of clinical records policies.



9.2 CMS GUIDANCE



9.3 OIG GUIDANCE

CMS STATE OPERATIONS MANUAL, APPENDIX B — GUIDANCE TO SURVEYORS: HOME HEALTH AGENCIES

Provides interpretive guidelines for survey of 42 CFR § 484.105 compliance; defines surveyor expectations for governing body evidence.

OIG COMPLIANCE PROGRAM GUIDANCE FOR HOME HEALTH AGENCIES (1998; SUPPLEMENTED)

Establishes expectation that the governing body actively oversees the compliance program and receives regular compliance reports.

CMS OASIS-E2 GUIDANCE MANUAL (EFFECTIVE APRIL 2026)

While not directly governing the Governing Body, the Governing Body is accountable for ensuring the agency's OASIS program meets CMS requirements (see CL-OA domain policies).



9.4 CROSS-REFERENCED AGENCY POLICIES

POLICY ID	POLICY TITLE	RELATIONSHIP
GV-OG-001	Organizational Structure & Reporting	Governing Body approves organizational structure.
GV-OG-002	Administrator Qualifications & Responsibilities	Governing Body appoints and evaluates Administrator.
GV-OG-003	Scope of Services Definition	Governing Body approves scope of services.
GV-OG-004	Strategic Planning & Annual Goals	Governing Body approves strategic plan.
GV-OG-005	Delegation of Authority	Governs limits of Governing Body delegation.
GV-PM-001	Policy Development & Approval Process	Governing Body approves REQUIRED-tier policies.
GV-PM-002	Policy Review & Revision Cycle	Governing Body ensures policy review cycle.
GV-PM-003	Policy Acknowledgment & Staff Attestation	Staff acknowledgment of this and all policies.
GV-GB-002	Board Meeting & Minutes Requirements	Details meeting documentation standards.
GV-GB-003	Conflict of Interest Disclosure	Governs member conflict disclosures.
GV-GB-004	Succession Planning for Key Leadership	Governing Body reviews succession plan.
GV-GB-005	Annual Governance Self-Assessment	Governing Body self-assessment tool.
GV-EA-004	Agency Licensure & Certification Maintenance	Governing Body ensures licensure/certification currency.
GV-EA-005	Agency Closure or Change of Ownership	Governing Body directs CHOW process.
QA-PG-001	QAPI Program Establishment & Governance	Governing Body oversees QAPI program.

POLICY ID	POLICY TITLE	RELATIONSHIP
QA-PG-002 	QAPI Plan Development & Annual Review	Governing Body approves QAPI plan.
QA-AE-003 	Corrective Action Plan Development & Tracking	Escalation path for governance deficiencies.
QA-SM-004 	Home Health Compare & Star Rating Monitoring	Data reported to Governing Body quarterly.
CO-CP-001 	Corporate Compliance Program	Governing Body oversees compliance program.
CO-CP-002 	Compliance Officer Designation & Authority	Governing Body appoints Compliance Officer.
CO-CP-005 	Whistleblower Protection & Non-Retaliation	Governing Body ensures non-retaliation.
CO-CP-007 	Compliance Investigation Process	Governing Body directs investigations.
CO-HP-007 	Record Retention & Destruction	Retention standards for governance records.
FN-FP-005 	Annual Budget & Financial Planning	Governing Body approves budget.
HR-TA-003 	OIG/SAM Exclusion Screening	Screening of Governing Body members.
OP-FM-005 	Emergency Operations & Business Continuity	Governing Body approves emergency plan.
EN-TG-001 	Enterprise Policy Taxonomy & Classification Governance	Framework under which this policy is classified.



10. TRAINING REQUIREMENTS

1. All Governing Body members of Care Indeed Home Health Care, Inc. shall receive orientation to this policy within 14 calendar days of appointment. Orientation shall be conducted by the Administrator or Compliance Officer and must cover: (a) the legal authority and responsibilities of the Governing Body; (b) meeting and quorum requirements; (c) conflict of interest obligations; (d) QAPI, compliance, and financial oversight expectations; (e) CMS survey process and surveyor expectations for governance documentation.
2. All Governing Body members and senior leadership personnel within scope of this policy (Section 3) shall sign the Policy Acknowledgment Form (Appendix C) within 14 calendar days of the policy effective date, any revision, or new appointment.
3. The Administrator shall maintain a tracking log of all policy acknowledgments and report any non-compliance to the Governing Body Chair within 7 calendar days of the acknowledgment deadline. Failure to acknowledge within the required timeframe shall result in written notification from the Governing Body Chair with a mandatory completion deadline of 7 additional calendar days.
4. Annual refresher training on governing body responsibilities shall be conducted at the first quarterly meeting of each calendar year. Attendance shall be documented in meeting minutes.



11. VERSION CONTROL

1. This policy is maintained under the agency's enterprise policy lifecycle management system per policy EN-LC-001.
2. Only the most current approved version of this policy, as reflected in the policy header, is valid for any operational, compliance, or regulatory purpose. All superseded versions must be archived and clearly marked as "SUPERSEDED — NOT FOR USE."
3. Any substantive revision to this policy requires: (a) review and approval by the Governing Body, documented in meeting minutes; (b) re-acknowledgment by all personnel within scope, within 14 calendar days of the revised effective date; (c) update to the enterprise policy index per EN-TG-001.
4. Non-substantive revisions (formatting, typographical corrections, updated cross-references) may be approved by the Administrator with notification to the Governing Body at the next regular meeting. Non-substantive revisions do not require re-acknowledgment.



APPENDICES

APPENDIX A — GOVERNING BODY MEMBERSHIP ROSTER

SECTION 1 — IDENTIFICATION

FORM COMPLETED BY (FULL NAME) *

TITLE / ROLE *

DEPARTMENT / BRANCH

DATE COMPLETED *

mm/dd/yyyy

SECTION 2 — TRACKING TOOL ENTRIES

MEMBER NAME	ROLE	VOTING STATUS	PRIMARY PHONE	EMAIL	EMERGENCY CONTACT	COMPETENCY AREA

SECTION — SIGNATURES & ATTESTATION

COMPLETED BY — PRINTED NAME

COMPLETED BY —
SIGNATURE

COMPLETED BY —
DATE

mm/dd/yyyy



SUPERVISOR / REVIEWER — PRINTED
NAME

SUPERVISOR /
REVIEWER —
SIGNATURE

SUPERVISOR /
REVIEWER — DATE

mm/dd/yyyy



APPENDIX B — CONFLICT OF INTEREST FORM

SECTION 1 — IDENTIFICATION

FORM COMPLETED BY (FULL NAME) *

TITLE / ROLE *

DEPARTMENT / BRANCH

DATE COMPLETED *

mm/dd/yyyy

SECTION 2 — ATTESTATION

I hereby certify that:

1.

The information provided is true, complete, and accurate to the best of my knowledge.
2.

I understand my ongoing obligation to disclose any new conflict within 7 calendar days of becoming aware.
3.

I will recuse myself from any deliberation or vote on matters in which I have a disclosed conflict.
4.

I have read and agree to abide by the Code of Conduct and Conflict of Interest Policy.

SECTION — SIGNATURES & ATTESTATION

DISCLOSING PARTY — PRINTED NAME

DISCLOSING PARTY
— SIGNATURE

DISCLOSING PARTY
— DATE

mm/dd/yyyy

APPENDIX C — POLICY ACKNOWLEDGMENT FORM

SECTION 1 — IDENTIFICATION

FORM COMPLETED BY (FULL NAME) *

TITLE / ROLE *

DEPARTMENT / BRANCH

DATE COMPLETED *

mm/dd/yyyy

SECTION 2 — LOG ENTRIES

MEMBER NAME	TRAINING TITLE	PROVIDER	DATE COMPLETED	HOURS	EVIDENCE

SECTION — SIGNATURES & ATTESTATION

COMPLETED BY — PRINTED NAME

COMPLETED BY —
SIGNATURE

COMPLETED BY —
DATE

mm/dd/yyyy



SUPERVISOR / REVIEWER — PRINTED
NAME

SUPERVISOR /
REVIEWER —
SIGNATURE

SUPERVISOR /
REVIEWER — DATE

mm/dd/yyyy



APPENDIX D — MEETING MINUTES TEMPLATE

SECTION 1 — IDENTIFICATION

FORM COMPLETED BY (FULL NAME) *

TITLE / ROLE *

DEPARTMENT / BRANCH

DATE COMPLETED *

mm/dd/yyyy



SECTION 2 — CONTENT

MEETING DATE *

mm/dd/yyyy



RECORDER *

MEMBERS PRESENT



MEMBERS ABSENT



QUORUM CONFIRMED? (Y/N)

— Select —



APPROVAL OF PRIOR MINUTES



DISCUSSION SUMMARY



MOTIONS / VOTES



ACTION ITEMS (OWNER / DUE)



ADJOURNMENT TIME

SECTION — SIGNATURES & ATTESTATION

CHAIR — PRINTED NAME

CHAIR —
SIGNATURE

CHAIR — DATE

mm/dd/yyyy



RECORDER — PRINTED NAME

RECORDER —
SIGNATURE

RECORDER — DATE

mm/dd/yyyy



APPENDIX E — QUARTERLY OVERSIGHT CHECKLIST

SECTION 1 — IDENTIFICATION

FORM COMPLETED BY (FULL NAME) *	TITLE / ROLE *
DEPARTMENT / BRANCH	DATE COMPLETED *
	mm/dd/yyyy

SECTION 2 — ASSESSMENT RESPONSES

SUBJECT / PATIENT / AREA ASSESSED

ASSESSMENT DATE

OVERALL RISK RATING

mm/dd/yyyy

— Select —

FINDINGS SUMMARY

STRENGTHS IDENTIFIED

OPPORTUNITIES FOR IMPROVEMENT

RECOMMENDED ACTION PLAN

FOLLOW-UP DATE

mm/dd/yyyy

SECTION — SIGNATURES & ATTESTATION

COMPLETED BY — PRINTED NAME

COMPLETED BY —
SIGNATURE

COMPLETED BY —
DATE

mm/dd/yyyy



SUPERVISOR / REVIEWER — PRINTED
NAME

SUPERVISOR /
REVIEWER —
SIGNATURE

SUPERVISOR /
REVIEWER — DATE

mm/dd/yyyy



APPENDIX F — ANNUAL CALENDAR

SECTION 1 — IDENTIFICATION

FORM COMPLETED BY (FULL NAME) *

TITLE / ROLE *

DEPARTMENT / BRANCH

DATE COMPLETED *

mm/dd/yyyy



SECTION 2 — CONTENT

MEETING DATE / TIME / LOCATION *

MEETING TYPE (REGULAR / SPECIAL / EXECUTIVE)

— Select —

CHAIR

INVITED ATTENDEES

AGENDA ITEM 1

AGENDA ITEM 2

AGENDA ITEM 3

AGENDA ITEM 4

AGENDA ITEM 5

ADDITIONAL ITEMS

SECTION — SIGNATURES & ATTESTATION

CHAIR — PRINTED NAME

CHAIR —
SIGNATURE

CHAIR — DATE

mm/dd/yyyy



APPENDIX G — AGENCY ORGANIZATIONAL CHART

SECTION 1 — IDENTIFICATION

FORM COMPLETED BY (FULL NAME) *

TITLE / ROLE *

DEPARTMENT / BRANCH

DATE COMPLETED *

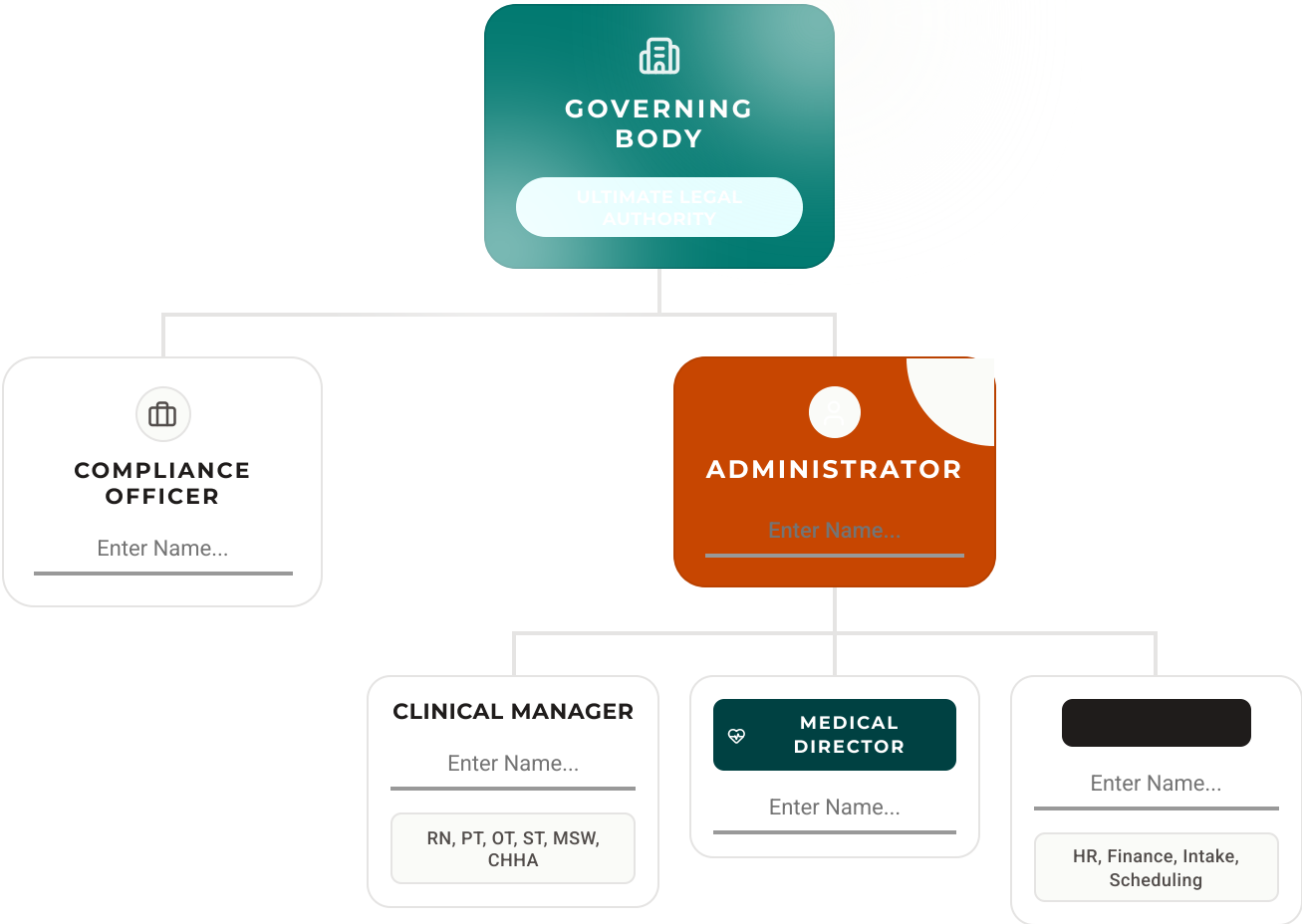
mm/dd/yyyy



SECTION 2 — WORKSHEET



Agency Organizational Structure: This chart illustrates the reporting relationships and accountability framework from the Governing Body through senior administrative and clinical leadership, as required by 42 CFR § 484.105.



SECTION — SIGNATURES & ATTESTATION

ADMINISTRATOR — PRINTED NAME

ADMINISTRATOR —
SIGNATURE

ADMINISTRATOR —
DATE

mm/dd/yyyy



GOVERNING BODY CHAIR — PRINTED
NAME

GOVERNING BODY
CHAIR —
SIGNATURE

GOVERNING BODY
CHAIR — DATE

mm/dd/yyyy

